

Digital Tech

2026

What you can achieve with no prior digital technology experience

In Feb 2026, I stood in the Surgical Assessment Unit in Chch Hospital with pent up frustration. Why are there so many people that don't know what medications they take? Why are they telling me they stopped their "brown round" pill because they didn't think it was important? They told me they changed their diabetes medications but really it was one for their heart. This made it incredibly difficult to accurately prescribe their meds, and to understand their level of knowledge around their health. Their adherence was poor and their autonomy was non-existent. They relied solely on us to get it right. This is not proactive healthcare.

So I had an idea - MedList. An app providing patients with, in Layman's terms, medication information AND brand names (because patients don't use the commercial names when talking to us). Most medication apps are clinician facing or just involve medication administration reminders but not user-friendly language and information. You can find out more about my app by visiting its site: <https://med-list.flutterflow.app/>

Now I did not know how on earth I was supposed to make something with no tech knowledge, so I consulted ChatGPT. This was before I knew there were many better AI LLMs. I told it my pain point and it told me how to produce the solution.

It started with making a list of medications by brand and commercial name, alongside their usage, side effects, organ system they help etc in Layman's terms. ChatGPT made me a CSV file of this and I kept adding to it until I had 430 medications. Now what I wanted was a constantly updating list that was accurate through NZ Formulary but their API was \$7500/yr. So for my MVP I used a manual database. At this stage I had the CSV file placed into Google Sheets as my database and I hooked it up to a no-code app builder, Glide. This was the first month of learning - how to create actions, UI, databases, privacy/IDs and medical disclaimers. This product works but I didn't understand the back-end at all because Glide handles all of that.

I also soon found out that although no-code app builders are amazing - they have their limits. The UI is crap, the ability to add APIs is minimal and it's expensive. It is \$60/month to use Glide. I also didn't feel like I was actually learning.

So first I moved on up to a better platform for my database where I could make it with SQL. It is called Supabase and essentially it uses Postgres SQL, allowing for real-time APIs, storage and authentication.

I used SQLBolt as the beginner level course to understand SQL basics. It allowed me to create my database that I initially had in Google Sheets. The reason this was useful was because SQL allows me to extract info and pick out trends from different tables. For e.g. in my app, each user has the number of meds they take on the UI by a simple SELECT query in SQL that gets projected and updated in real-time. Supabase itself allows for a better overview of API (Application programming interface) and Row-Level Security (restricts data access to specific rows based on user identity, which is very important for health information). Still talking to ChatGPT, it got me to learn how to write RLS and connect up APIs to app-builders and eventually other LLMs.

I moved from using Glide to another low-code app builder called Flutterflow and this was purely as it has more creative freedom. More API integration, better UI design, allows me to integrate PDFs etc. I started to understand app architecture. So my second MVP is the same app (MedList) but on flutterflow. This is where I got Claude to write me a code for a PDF edge-function.

Using my clinical experience, I thought it would be very useful to have users open the app and generate a PDF of their health profile and updated medication. Getting this to work meant knowing JSON and edge functions. My PDF had to be constantly extracting info from Supabase's database for each individual user and nicely formatting it into a sheet. This took me days to learn how to do and many errors.

There are still things I cannot do with LLM assistance, like how to upload pictures on the app so that's something to learn in the future.

Moving on from here I had a second idea for an app that was inspired by a close friend of mine. This is where I learned about AIs - LLMs, agentic AI, tokens, context windows, API keys, prompt architecture, evals, hallucinations and the like.

The first thing I did for my next app, SignalHealth, is write a prompt to try out on my browser, on Gemini, Claude and ChatGPT. I would copy/paste it in every conversation and tell the LLM to act as that prompt. I would stress test it over and over to make it more clinically credible and safe. I wanted the app to inform users when to see a doctor but not to diagnose, to escalate when things seemed serious but also reassure and not over-escalate. I modified the tone of the AI, the way it onboarded a conversation, the way it was formatted, then started adding in my clinical knowledge. This included writing the specific red-flag symptoms like chest pain, blindness etc. Then writing a medication safety tool such as potential side effects, contraindications like combining drugs. I added in distinguishing between chronic illness and acute issues and so on.

The prompt is now 20 pages long. To understand more about what SignalHealth actually is, visit the app <https://signalhealth.dev/login> . It is an AI health companion.

Only in the last week have I learned of Promptfoo which is an open-source CLI and library used to test, evaluate, and secure Large Language Model (LLM) applications. So I will write in the different things I want to test my prompt on and then Promptfoo will do multiple iterations and stress-test it and come back with a pass/fail, some feedback etc. It is now built into my code so I can run it whenever I want.

At this stage I was in a rut because SignalHealth did not carry memory. This is where I learned about LLMs having context but not real memory. Conversation windows had a specific number of tokens and eventually as they run out, the chat becomes less accurate, more forgetful and starts hallucinating essentially. This is dangerous for a health app. So I had to link it to a database and store the conversation over time so it was holding onto knowledge.

How did I do this? Initially I set up Supabase and made my tables, for e.g. one is a health profile for a user (it had their name, meds, ethnicity, family history, allergies etc). There was a health story column which basically collated the conversation history over time and summarised it into a nice patient narrative. To do all of this I got Claude to write me the code and I pasted it into VSC (Visual Studio Code). VSC is a command-line interface where you can write code and install programmes.

I hadn't used VSC before or used json. But Claude helped me download [node.js](https://nodejs.org/) and set it up in VSC. And I started making my app. I created API keys to Supabase, put in my chatbot prompt, had pages for my UI, a GP summary page, and API keys to Anthropic as my chatbot's LLM. I started to get how it all works together. Slowly Claude writing the code for me was inefficient because our conversation was too long and it started making mistakes.

My friend introduced me to Cline. "An autonomous AI coding agent designed to help you build, edit, and manage complex software development projects directly from your code editor or terminal". Cline works in VSC and can read my code, and change it when needed with my approval. It teaches me how things work too. I didn't need to use Claude anymore to copy/paste code.

I slowly realised as my code increased that if I lost it all, I'd be screwed. My whole app would be lost...until I found such a thing known as CI/CD (Continuous integration and deployment). At this stage in my journey I'd set up Github which is pretty much a dynamic CV for tech people. It has all of my projects on there, my methods and shows how I got to where I am now. In VSC I started pushing everything to Github so my code was always backed up and there to see. From there it is automatically deployed in Vercel which is a "cloud platform that helps developers build, deploy, and scale web applications quickly". I didn't need Flutterflow as Vercel just handled the front-end of my code and I could deploy the app as I went.

Now one thing that I encountered an issue with is cost. When making a clinical product never forget the cost of using AI. For every user on the app, for every message sent, every profile extraction made etc - tokens are used. It stacks up quickly...so this is where I learned how to optimise cost but still keep memory accurate. I read more about prompt caching, batching and model cascading etc. Unfortunately I am still struggling with finding the balance here with medical safety and financial security for myself as I am the only one building the app.

But it is very valuable to learn about this because the cost of a product is significant in making decisions.

I now have the SignalHealth app up and running but it needs lots of work. The prompt needs to be optimised with more stress-tests, the memory needs to be checked and I have still not managed to figure out creating notifications. This is the other feature I want, where SignalHealth comes to you and checks-in autonomously. This is where I could use the hands of some software engineers.

Once this is done I will move onto user research, and testing it out.

To make these apps into real products and get funding - I needed a business plan. This laid out why my apps are needed in the health world. It explained how my apps were different from the current market. It explained the risks and things to improve. The benefit to the overall system etc.

Building a product takes more than just an idea. It takes consistent trial-and-error and thinking "how will I get this to work" and just doing it. In a time where we have AI, this is so valuable.

To work in healthtech the best thing you can do as a clinician, is just find a pain point and try to solve it. Create a project and a folio. Companies want to see that you can deliver and not just imagine. They want to see how your clinical experience can be an asset for the company.

I am taking a huge risk by trying to make SignalHealth a scalable product but if it doesn't succeed, at least I now have the experience to transition into health-tech in other ways.

So reflecting on this, in the last 3 months I have learned SO MUCH and this is only by doing.